

Our Ref: (T) GB/TXW DERM/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Dermatology

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , Flat [ADDRESS], [ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnosis: Androgenic alopecia.

Treatment Plan:

To continue with iron supplements

Minoxidil 2% to be tried

British Association Dermatologist Leaflet given on androgenetic alopecia

Follow-up: Discharge.

[NAME] was reviewed in clinic today. [NAME] recently attended for our clinical dermatology me

[NAME] was reviewed by my other consultant colleagues. Clinically her hair loss was consistent

female pattern hair loss. Her histology slides were reviewed by Dr [NAME] who suggested that

of alopecia was not consistent with scarring alopecia but did show changes of female pattern

This diagnosis was discussed in detail with [NAME] today and I have given her a British Assoc

Dermatologist leaflet on androgenetic alopecia. I have suggested [NAME] tries Minoxidil 2% in

if [NAME] is is able to give it a reasonable try [NAME] should try for at least four to six m

[NAME] can subsequently move on to 5% if [NAME] wishes to however there is not much evidence

difference in efficacy between the two concentrations.

[NAME] should also continue with iron supplements. I have not arranged to see her again routi

[NAME] has now been discharged back to your care.

With kind regards

Yours sincerely

[NAME]

[NAME]

[DICTATION-META]

[PROVIDER-NAME], FRCP(UK), MRCP (Med), MRCP (Derm)

Consultant Dermatologist & Cutaneous Surgeon